



The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment.
Global data context, 2023.

Source: Our World in Data (based on IHME, Global Burden of Disease)

Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major Depression

1 in 3

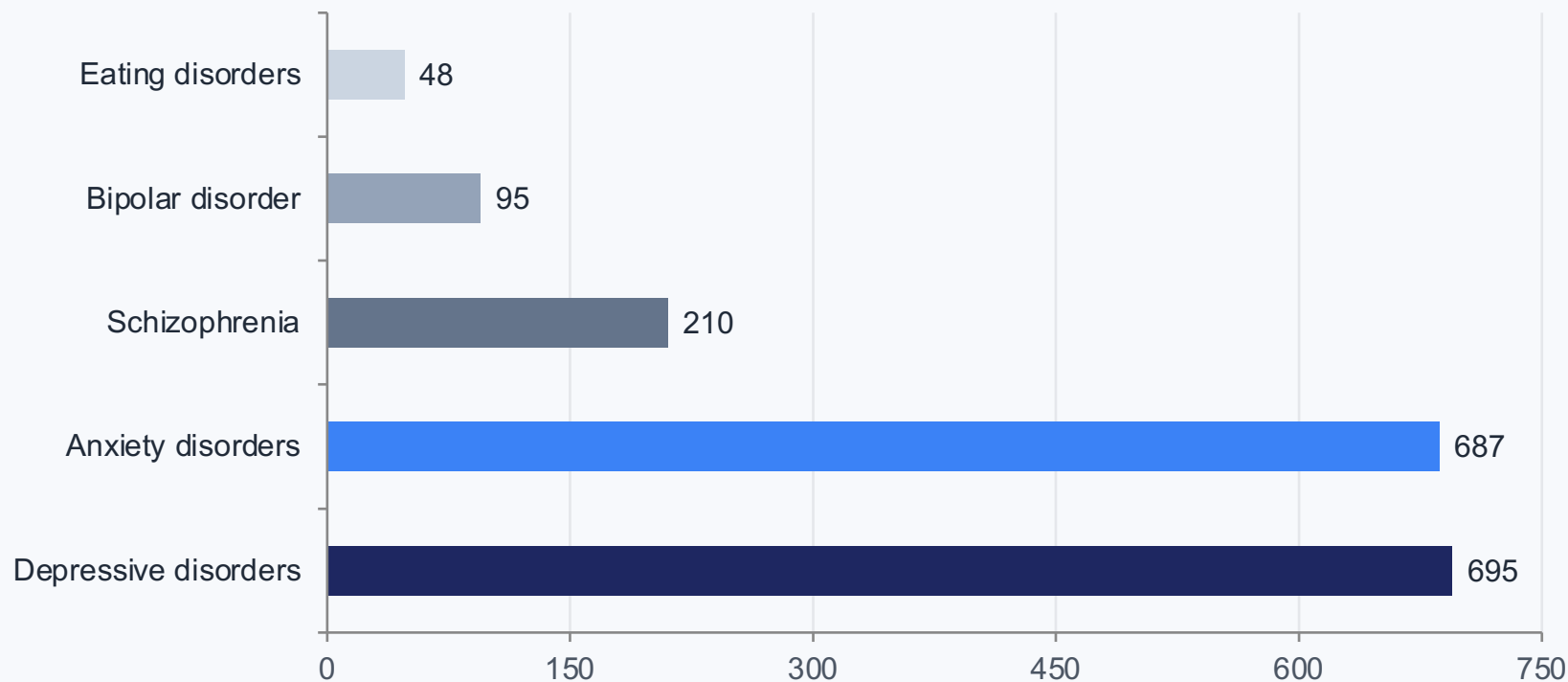
women experience major
depression
at some point in life

1 in 5

men experience major depression
at some point in life

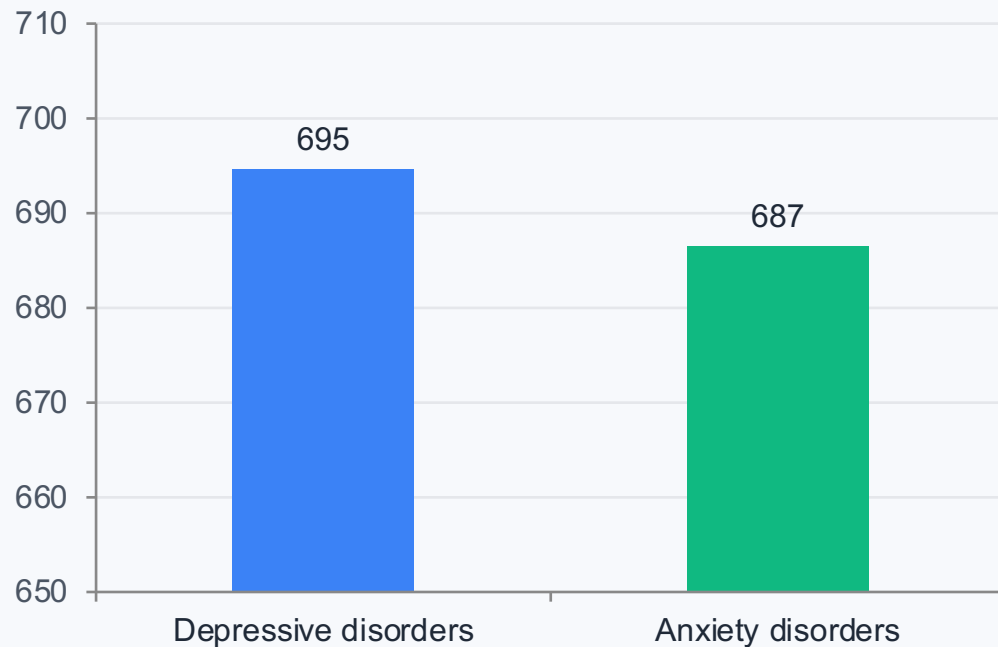
Mental illnesses include common high-burden disorders (depression, anxiety) and less common but severe conditions (schizophrenia, bipolar disorder), requiring differentiated policy responses.

Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respectively



DALYs per 100,000 people, by mental illness category (Figure 1 reference).

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

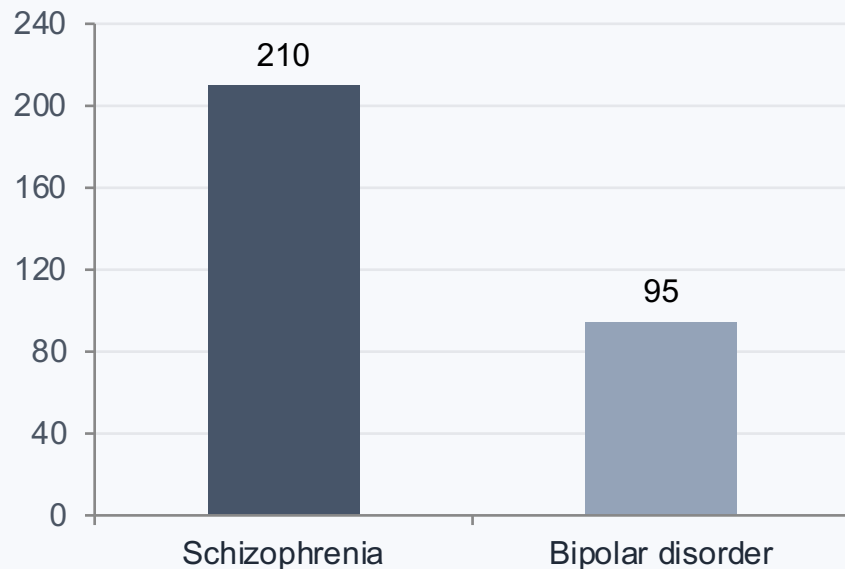


Near-Equal Burden

Difference:
8.1 DALYs
per 100,000

Together these two categories account for the vast majority of measured mental health DALY burden, far exceeding other categories.

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact



Schizophrenia: 210.2 DALYs per 100,000 | Bipolar disorder: 94.6 DALYs per 100,000

Policy Implication

Lower population DALYs than depression/anxiety

But high disability and severity for affected individuals

Specialized care pathways must be maintained while scaling common-disorder services

Treatment Is Available but Often Lacking or Poor Quality



Stigma

People may avoid disclosing symptoms to professionals or people they know.



Access

Care is often unavailable, especially in low- and middle-income countries.



Quality

Even when services exist, treatment quality may be inadequate.



Underreporting

Reluctance to disclose symptoms can make true prevalence hard to measure.

Mental illnesses are treatable, but systemic barriers create a persistent treatment gap and likely underestimation of true burden.

How People Cope: From Medication to Faith to Family Conversations



Prescribed Medication

Clinical treatment through
prescribed medicines.



Religious or Spiritual Activities

Faith-based coping and spiritual
practices.



Talking with Friends/Family

Informal social support and
conversation.

Coping strategies vary widely by country and culture, indicating that policy and programs should be culturally sensitive and context-specific (Figure 2 reference).

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

- Depression and anxiety represent the overwhelming majority of mental health DALYs (~695 and ~687 per 100,000).
- Schizophrenia and bipolar disorder contribute lower population DALYs but carry high individual severity and disability.
- Effective treatment exists, yet access and quality remain inadequate in many settings.
- Stigma suppresses help-seeking and weakens prevalence measurement through underreporting.
- Policy should scale evidence-based interventions for common disorders while sustaining specialized care for severe conditions.

Action focus: scale population-level services for common disorders, preserve specialist pathways for severe mental illness.